This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 30 SEPTEMBER 2026

Patient Group Direction

for the administration of Rabipur or Verorab (Rabies Vaccine) for pre-exposure prophylaxis against rabies

Pre-exposure Prophylaxis against Rabies

From age 2 years onwards.

Patient Group Direction, version 008, issued 30 September 2026. Rabipur and Verorab, intramuscular route only, pre-exposure only.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

The administration of rabies vaccine, before any exposure, to a traveller or worker aged 2 years or over who is at risk of contact with rabies, by a registered pharmacist or registered pharmacy technician in a community pharmacy, without a prescription.

What it authorises

- Rabipur or Verorab by the intramuscular route: three doses on days 0, 7 and 21 to 28, or, where there is not time before travel, three doses on days 0, 3 and 7 with a further dose at one year.

What it does not cover

- Treatment after a bite, scratch or lick. That is post-exposure treatment, which this PGD does not cover. Send the patient to urgent care the same day.
- A patient who has had a confirmed anaphylactic reaction to a previous dose or to any component of the vaccine.
- Children under 2.

Summary of Green Book guidance for rabies pre-exposure vaccination

Rabies, Immunisation against infectious disease (the Green Book), chapter 27, dated 19 May 2023 (page last updated 26 May 2023). Summarised 9 September 2026.

The disease, and why pre-exposure vaccination matters

Rabies is an acute viral encephalomyelitis caused by lyssaviruses, either rabies virus genotype 1 (classical rabies) or, less commonly, rabies-related lyssaviruses such as European bat lyssaviruses and Australian bat lyssavirus.

Infection is usually via the bite or scratch of a rabid animal, most frequently a dog. Transmission has rarely occurred when body fluids from an infectious animal contact mucous membranes. Infection does not occur through intact skin.

The incubation period is generally three to 12 weeks but may range from four days to 19 years, and in more than 93% of patients onset is within one year of exposure.

RABIES IS ALMOST ALWAYS FATAL. There is no specific treatment other than supportive care once clinical symptoms develop, and death results from respiratory paralysis. An estimated 59,000 people die of rabies each year worldwide.

Deaths in the UK continue to occur in people infected abroad, none of whom had received timely and appropriate post-exposure treatment.

Intramuscular immunisation with tissue culture vaccines reliably produces rabies virus neutralising titres in approximately 95% of recipients, and an immunocompetent person who has had a primary course is primed and responds promptly to a booster.

PRE-EXPOSURE IMMUNISATION DOES NOT REMOVE THE NEED FOR POST-EXPOSURE TREATMENT. A primed individual responds more rapidly and in most cases does not require human rabies immunoglobulin, but a fully immunised person with a significant exposure still needs two doses of vaccine, on days 0 and 3 to 7.

Who should have pre-exposure vaccination

The need for pre-exposure immunisation is decided by an individual risk assessment of potential exposures.

Within the UK: laboratory staff routinely working with rabies virus, workers at DEFRA-authorised quarantine premises and carriers, those who regularly handle bats including on a voluntary basis, and veterinary and technical staff who by reason of their employment encounter enhanced risk.

Outside the UK: animal control and wildlife workers, veterinary staff and zoologists who regularly work in rabies enzootic areas.

Travellers to rabies enzootic areas should be offered pre-exposure immunisation, especially where post-exposure medical care and rabies biologics at the destination are lacking or in short supply, where they are undertaking higher risk activities such as cycling or running, or where they are living or staying for more than one month.

Country-specific travel risk is available from the National Travel Health Network and Centre, and in Scotland from Public Health Scotland Travax or FitForTravel.

Under the Health and Safety at Work Act 1974 and the COSHH Regulations 2002, employers must assess the risk of exposure to rabies virus and protect employees so far as is reasonably practicable, including providing pre-exposure immunisation where indicated.

UKHSA does not supply pre-exposure rabies vaccine for travellers. For pre-exposure indications other than bat handlers with no identifiable employer, the vaccine is obtained through local pharmacies by private prescription, which is the service this PGD supports.

The vaccines, and the pre-exposure schedule

The chapter states that there is currently one rabies vaccine licensed for intramuscular use in the UK, purified chick embryo cell rabies vaccine (PCEC, 2.5 IU/mL), Rabipur, and that Rabies Vaccine BP (human diploid cell vaccine) from Sanofi was discontinued in 2022. This is out of date: see the note below on Verorab, UK-licensed since November 2023.

Rabipur contains 2.5 IU rabies antigen in a 1 mL dose, is thiomersal-free and inactivated, contains no live organism and cannot cause the disease against which it protects. It is a freeze-dried suspension of the Flury LEP-25 strain cultured in chick embryo cells and inactivated with betapropiolactone, and contains traces of amphotericin B, chlortetracycline and neomycin.

Other WHO approved cell culture derived vaccines are available in other countries and may contain different concentrations of rabies antigen. Those products should be used as recommended by their manufacturer. Cell culture derived rabies vaccines may be used interchangeably.

Rabipur is supplied as a freeze-dried powder with solvent. After reconstitution it should be used immediately and no later than ONE HOUR after reconstitution.

STANDARD PRIMARY COURSE: three doses (2.5 IU, one vial) intramuscularly on DAYS 0, 7 AND 28. The third dose can be given from day 21 where there is insufficient time before travel. Where there is time to complete the 21 to 28 day course, this is the preferred schedule.

ACCELERATED COURSE, where there is insufficient time before travel: three doses intramuscularly on DAYS 0, 3 AND 7, with an ADDITIONAL DOSE AT ONE YEAR if the person will continue to travel to high risk areas.

Give intramuscularly into the upper arm or anterolateral thigh. For an individual with a bleeding disorder, give by deep subcutaneous injection to reduce the risk of bleeding.

JCVI recommends the intramuscular rather than the intradermal route for pre-exposure prophylaxis, and recommends that only the intramuscular route, or the deep subcutaneous route for those with bleeding disorders, is used for post-exposure treatment.

Rabies vaccine can be given at the same time as other vaccines, including other travel vaccines, at separate sites and preferably in different limbs, and if in the same limb at least 2.5 cm apart. Record the site of each.

THE VACCINEE MUST KEEP A RECORD of the vaccine and regimen received, because it will determine their post-exposure treatment.

Boosters and antibody testing

The need for boosters depends on the indication for pre-exposure prophylaxis and the likely frequency of ongoing exposures.

Where unrecognised exposures may be frequent, for example bat handlers, give a single reinforcing dose one year after the primary course, then boosters every three to five years or based on serology.

Laboratory staff routinely working with rabies virus should have vaccine antibody tested at six monthly intervals to determine the timing of boosters. Antibody levels of at least 0.5 IU/mL are considered protective.

ROUTINE BOOSTERS ARE NOT RECOMMENDED FOR MOST TRAVELLERS. A single booster can be considered, following a risk assessment, for someone who completed a primary course over one year ago and is travelling again to a high risk area.

A complete primary course is three doses over 21 to 28 days, or an accelerated three dose course over 7 days plus an additional dose at one year.

Contraindications and precautions

Do not give pre-exposure rabies vaccine to anyone who has had a confirmed anaphylactic reaction to a previous dose of rabies vaccine or to any component of the vaccine.

Rabipur contains residues of chicken proteins such as ovalbumin, so an alternative rabies vaccine may be considered for PRE-EXPOSURE immunisation in someone with severe egg allergy.

Minor illness without fever or systemic upset is not a reason to postpone. If the individual is acutely unwell, postpone until recovery, so that signs or symptoms of the illness are not wrongly attributed to the vaccine.

Where a hypersensitivity reaction to a dose of a pre-exposure course has occurred, the individual should still receive POST-exposure immunisation if indicated, because the risk of rabies outweighs the risk of hypersensitivity. Seek specialist advice and give further doses under close medical supervision.

THERE ARE NO CONTRAINDICATIONS TO POST-EXPOSURE TREATMENT, including Rabipur in severe egg allergy. Post-exposure treatment should always be started without delay, even in the presence of other illness.

Those with immunosuppression and HIV infection, regardless of CD4 count, may be given pre-exposure vaccine as above but may not make a full antibody response. Re-immunisation should be considered after treatment is finished and recovery has occurred, or after immune recovery on antiretroviral treatment.

The intradermal 0.1 mL pre-exposure regimens should not be used in those who are immunosuppressed or taking chloroquine for malaria prophylaxis, because the antibody response can be suppressed. Chloroquine does not suppress the response when the vaccine is given intramuscularly, which is the route this PGD authorises.

Pregnancy and breastfeeding

Pregnant women and breastfeeding mothers should be given PRE-exposure prophylaxis if, following a risk assessment by a health professional, the risk of exposure is high and rapid access to post-exposure treatment would be limited.

POST-exposure treatment should be given to pregnant women and breastfeeding mothers when indicated.

Adverse reactions

Local reactions such as redness, swelling or pain at the injection site may occur within 24 to 48 hours.

Systemic reactions such as headache, fever, muscle aches, vomiting and urticarial rashes are rare.

Delayed hypersensitivity reactions have been reported from the United States, and reactions may become more severe with repeated doses.

Neurological conditions such as Guillain-Barre syndrome have been reported extremely rarely, and a causal association with immunisation is not established.

Report all suspected adverse reactions to the MHRA through the Yellow Card scheme at https://yellowcard.mhra.gov.uk/.

What the traveller must be told to do if bitten, licked or scratched

Every traveller to an enzootic area must be told the practical steps to take if bitten, or if saliva from an infected animal contacts broken skin or mucous membranes such as the eyes, nose or mouth.

AS SOON AS POSSIBLE, clean the wound by flushing under a running tap for several minutes and washing with soap or detergent and water.

Then apply a suitable disinfectant and cover with a simple dressing. Suitable disinfectants include 40 to 70% alcohol, or tincture or aqueous solution of povidone-iodine.

Wash salivary exposures to the eyes, nose or mouth thoroughly with clean water as soon as possible.

AVOID PRIMARY SUTURE, or postpone it until post-exposure treatment has started, because it can cause further damage and may increase the risk of introducing virus to the nerves.

SEEK MEDICAL ATTENTION so that a full risk assessment can be done and post-exposure treatment started promptly if indicated, taking account of the country, the animal species and health status, the category and site of exposure, and the person immune status and previous rabies vaccination.

Rabies can be transmitted by the saliva of any warm blooded animal, including bats, monkeys and rodents as well as cats, dogs and foxes. A normal appearance and behaviour of the animal does NOT exclude rabies. Bat bites from UK species are usually felt and not seen.

Because the incubation period can be prolonged, a risk assessment should still be done and treatment still considered even if the exposure was many months or years earlier.

Specialist advice: in England, the UKHSA Rabies and Immunoglobulin Service on 0330 128 1020 or the local health protection team; in Wales, the Public Health Wales All Wales Acute Response Service on 0300 003 0032; in Northern Ireland, the Public Health Agency Duty Room on 030 0555 0119; in Scotland, the local on-call infectious diseases or infection specialist.

The Patient Group Direction

How to use this PGD

Two schedules, and which one you may use

- CONVENTIONAL COURSE, PREFERRED, ALL AGES: three doses on day 0, day 7 and day 28. The third dose may be brought forward to day 21 where there is insufficient time before travel.
- ACCELERATED COURSE, 18 YEARS AND OVER ONLY: three doses on day 0, day 3 and day 7, with a further dose at one year if travel to high risk areas continues. Use only where there is genuinely insufficient time for the conventional course.
- The accelerated course is OFF-LABEL and requires documented consent. See the row below. Neither product SPC contains a three dose day 0, 3 and 7 pre-exposure regimen.
- Under 18s may have the conventional course under this PGD. They may NOT have the accelerated course. Where a child needs a fast course, refer to a travel clinic or the GP.
- Do not use the two dose day 0 and day 7 regimen that now appears in both product SPCs. UKHSA and JCVI guidance on its use in the UK is still awaited. This PGD follows the Green Book three dose schedules until that position is confirmed and this document reissued.
- Observe every patient for 15 minutes after vaccination.
- THIS IS PRE-EXPOSURE PROPHYLAXIS ONLY. Any bite, scratch or lick on broken skin that has already happened is a post-exposure situation and a same-day medical emergency. Refer; do not manage it here.

Where the age restriction comes from

The 18 year floor on the accelerated course is a Get Real Health service restriction. It is recorded here plainly so that nobody later mistakes it for a guideline requirement.

- Green Book chapter 27 (19 May 2023) sets no age limit on the accelerated course.
- The Verorab SPC records that its one-week pre-exposure regimen was studied in 75 participants including 35 children aged 2 to 17, all of whom reached protective titres.
- Get Real Health has nevertheless restricted the accelerated route to adults, on the basis that a compressed course in a child with a fixed travel date leaves no margin if a dose is missed, and that a child needing a fast course is better seen by a travel clinic.
- A pharmacist should not read this restriction as a contraindication. It is a scope decision, and it may be revisited.

A note on the Green Book and Verorab

Green Book chapter 27 states that Rabipur is the only rabies vaccine licensed for intramuscular use in the UK. That chapter was published on 19 May 2023. Verorab received its UK marketing authorisation, PLGB 23228/0001, on 1 November 2023, and its SPC was last revised on 29 January 2026. Both products in this PGD are UK-licensed. The chapter is simply out of date on this point, and a pharmacist who reads it should not conclude that Verorab is an unlicensed import.

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Patient Group Direction

for the administration, for pre-exposure prophylaxis against rabies, of:

Rabies vaccine, pre-exposure prophylaxis

Patient Group Direction for the administration of Rabipur or Verorab by the intramuscular route for pre-exposure prophylaxis against rabies.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be trained to the National Minimum Standards and Core Curriculum for Immunisation Training, and be competent in intramuscular vaccination technique.
- All users must be competent in the recognition and immediate management of anaphylaxis, and hold current basic life support training.
- All users must be competent in assessing consent in children and young people, including parental responsibility and Gillick competence. The conventional course under this PGD is available from 2 years of age, so children are in scope.
- All users must know that the two products have DIFFERENT DOSE VOLUMES: Rabipur 1.0 mL, Verorab 0.5 mL. Confirm which product is in hand before drawing up.
- All users must understand that the accelerated day 0, 3 and 7 course is off-label under both SPCs and must be able to explain that to a patient and record the consent.
- All users must be able to distinguish a pre-exposure request from a post-exposure one, and must treat any exposure that has already occurred as a same-day emergency.
- All users must be competent in vaccine handling and cold chain management, including the action to take on a cold chain excursion.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Pre-exposure prophylaxis against rabies in individuals at increased risk through travel or occupation, in accordance with Green Book chapter 27. Rabies is an almost invariably fatal viral encephalitis once symptoms develop.

	Pre-exposure vaccination does not remove the need for post-exposure treatment, but it simplifies that treatment considerably and removes the need for rabies immunoglobulin, which is expensive and often unavailable in the countries where the risk is highest.
Inclusion criteria	● Aged 2 years and over for the CONVENTIONAL course. Both products are licensed in all age groups; the 2 year floor is a Get Real Health service restriction. ● **18 years and over for the ACCELERATED course.** ● Travel to a rabies enzootic area, particularly where post-exposure treatment and rabies biologics are lacking or in short supply at the destination, or the trip involves higher risk activities such as cycling or running, or the stay is longer than one month. ● Occupational risk abroad: animal control and wildlife workers, veterinary staff and zoologists working in enzootic areas. ● UK-based occupational risk: laboratory staff working with rabies virus, workers at DEFRA-authorised quarantine premises or animal carriers, those who regularly handle bats including voluntarily, and veterinary and technical staff with enhanced occupational risk. ● Sufficient time before travel to complete the chosen course. ● **Valid informed consent obtained. Where the individual is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis of that assessment recorded.** ● **Where the accelerated course is used (18 years and over only), documented consent to off-label use.** ● No exclusion criterion present.
The accelerated course is off-label: what to say and record	**Neither SPC contains a three dose day 0, 3 and 7 pre-exposure regimen. Rabipur and Verorab each specify a three dose day 0, 7, 21 or 28 course, and a two dose day 0 and day 7 one-week regimen. The day 0, 3 and 7 course used here comes from Green Book chapter 27, not from either product licence, and is therefore off-label.** **Where the accelerated course is used, tell the patient all of the following and record that you did:** ● That the schedule being used is recommended by UK national guidance but is not the schedule in the manufacturer's licence. ● That the conventional day 0, 7 and 28 course is the preferred one, and is being departed from only because there is not enough time before travel. ● That a further dose will be needed at one year if travel to high risk areas continues. ● That pre-exposure vaccination does not remove the need to seek urgent treatment after any bite, scratch or lick on broken skin. **Record the consent as given for off-label use, naming the schedule. A general consent to vaccination is not sufficient.** *This mirrors the handling of the accelerated Japanese encephalitis schedule in the Japanese encephalitis PGD.*
Exclusion criteria	**Refer, do not vaccinate, where any of the following applies.** ● **ANY actual or possible exposure that has already occurred, including any bite, scratch or lick on broken skin from a mammal in a rabies risk area, however trivial and however long ago. This is post-exposure. Refer for urgent medical assessment the same day.** ● **Under 18 and the accelerated course is being requested. The conventional course may still be given.**

	• **Aged under 2 years. This is a Get Real Health service restriction; refer to the GP or a travel clinic.** • Confirmed anaphylactic reaction to a previous dose of rabies vaccine or to any component of the product to be used. • **For Rabipur specifically: severe egg allergy, because it contains chick embryo cell residues including ovalbumin. Verorab may be a suitable alternative.** • **Hypersensitivity to neomycin excludes BOTH products: Rabipur contains traces of neomycin, chlortetracycline and amphotericin B, and Verorab may contain traces of polymyxin B, streptomycin and neomycin. For Verorab specifically: hypersensitivity to polymyxin B or streptomycin, or to any antibiotic of the same class.** • Acute severe febrile illness. Postpone until recovered. A minor illness without fever is not a reason to defer. • Intradermal administration. This PGD covers the intramuscular route only. The intradermal route requires specific technique and is outside its scope. • **Immunosuppressed and the accelerated course is being requested. The Verorab SPC states the one-week regimen should not be used in immunocompromised individuals, and the same caution applies here. Use the conventional course and refer for post-course serology.** • Valid informed consent not obtained, or where the accelerated course is used, consent to off-label use not obtained.
Cautions	Pregnancy. Give pre-exposure vaccine where the risk of exposure is high and rapid access to post-exposure treatment would be limited, and record the risk assessment. There is no identified harm signal but human data are limited. Breastfeeding. The same principle applies. No risk to the infant has been identified. **Immunosuppression, including HIV. A full response may not be mounted. Use the conventional three dose schedule rather than the accelerated one, and refer for post-course serology to confirm a protective titre, taken as 0.5 IU/mL or above.** Bleeding disorders, thrombocytopenia or anticoagulation. Give by deep subcutaneous injection rather than intramuscularly. Chloroquine antimalarial prophylaxis suppresses the response to intradermally administered rabies vaccine. This PGD uses the intramuscular route only, which is not affected, but the interaction is recorded here because patients may ask. Where rabies vaccine is co-administered with Japanese encephalitis vaccine on the accelerated JE schedule, a faster decline in rabies antibody has been observed. Bear this in mind when advising on booster timing for a traveller who has had both. Verorab contains phenylalanine, 4.1 micrograms per 0.5 mL dose. Relevant only in phenylketonuria, and at that quantity almost certainly immaterial, but worth knowing if asked. **Verorab prefilled syringes without an attached needle have a tip cap containing a natural rubber latex derivative. Check before use in a latex-sensitive patient.**
Anaphylaxis and observation	**Adrenaline (epinephrine) 1 in 1,000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance, and a telephone.** All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life

	support training. **OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION. Vaccinate seated.** Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection. Procedures must be in place to prevent injury from a faint, which is common particularly in adolescents.
Actions if the patient is excluded or declines	● Discuss the reason for exclusion with the patient and make sure they understand it. ● Advise on alternative options and how to access them: the GP, a travel clinic, or a specialist service. ● **Where the exclusion is a possible exposure that has already occurred, make the urgency explicit. That patient needs assessment today, not an appointment.** ● **Where a child needs a fast course and cannot have the accelerated schedule here, say so plainly and direct them to a travel clinic rather than leaving them without a route.** ● Document the reason for exclusion, the advice given and the decision reached. Inform or refer to the GP as appropriate.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	● **That valid informed consent was given, and from whom. Where the individual is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment.** ● **Where the accelerated course was used, that consent to off-label use was given, naming the schedule.** ● Patient name, address, date of birth, and the GP with whom they are registered. ● **Which product was given, Rabipur or Verorab, with batch number and expiry date.** ● **Which schedule was used and why, and where accelerated, the reason the conventional course was not possible.** ● **Dose number in the course, and the date the next dose is due.** ● Date of administration, dose volume, route and anatomical site. ● Destination and reason for travel, or the occupational indication. ● Name and registration number of the healthcare professional administering. ● **That the 15 minute observation period was completed.** ● Advice given, including advice given if the patient is excluded or declines. ● Details of any adverse drug reactions and the actions taken. ● That the vaccine was administered under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when the course finished. The patient must also be given a written record, because it determines their post-exposure treatment if they are ever bitten.

The medicine

Name, form and strength	● RABIPUR (Bavarian Nordic), purified chick embryo cell vaccine. Powder and solvent, reconstituted to 1.0 mL per dose. Thiomersal free. Contains chick embryo cell residues and traces of neomycin,

	chlortetracycline and amphotericin B. ● VERORAB (Sanofi), purified Vero cell rabies vaccine. Powder and solvent, reconstituted to 0.5 mL per dose. PLGB 23228/0001. May contain traces of polymyxin B, streptomycin and neomycin. ● **THE DOSE VOLUMES DIFFER: 1.0 mL for Rabipur, 0.5 mL for Verorab. Confirm which product is in hand before drawing up. Do not carry a volume across from one product to the other.** ● Where a course is started with one product, complete it with the same product wherever possible. The cell-culture vaccines may be used interchangeably if necessary.
Legal category	POM.
Dose and frequency	● CONVENTIONAL, preferred, all ages: day 0, day 7 and day 28. The third dose may be brought forward to day 21 where time before travel is short. ● **ACCELERATED, 18 and over only, off-label, documented consent required: day 0, day 3 and day 7, with a further dose at one year if travel to high risk areas continues.** ● Boosters are not routinely recommended for most travellers. A single booster may be considered after risk assessment where the primary course was completed more than a year ago and the individual is travelling again to an enzootic area. ● Those with frequent unrecognised exposure risk, such as bat handlers, should have a reinforcing dose at one year and then boosters every 3 to 5 years, or as guided by serology. ● Laboratory staff working routinely with live virus should have antibody titres checked every 6 months to determine booster timing. This is outside pharmacy scope; refer to occupational health. ● **DO NOT use the two dose day 0 and day 7 regimen that appears in both SPCs. UKHSA and JCVI guidance on its use is awaited and this PGD deliberately follows the Green Book three dose schedules until that position is confirmed and this document reissued.**
Quantity to be administered	One dose per patient per attendance, in accordance with the schedule above. This PGD does not permit supply of vaccine to the patient for administration elsewhere.
Route and method of administration	Intramuscular, into the deltoid in older children and adults, or the anterolateral thigh in young children. Never into the buttock. For a patient with a bleeding disorder, give by deep subcutaneous injection instead. Reconstitute immediately before use. Rabipur must be used within one hour of reconstitution; Verorab for intramuscular use must be used immediately. Where given with other vaccines, use separate sites, preferably different limbs, or at least 2.5 cm apart, and record the site of each.
Storage and cold chain	Store at +2C to +8C in the original packaging to protect from light. Do not freeze; discard if frozen. **COLD CHAIN EXCURSION: vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use. Do not administer excursion stock until that assessment is complete.**
Disposal	Dispose of used syringes, needles, vials and any reconstituted vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Adverse effects	Very common: headache, malaise, myalgia, injection site pain, fatigue and fever. Common: lymphadenopathy, reduced appetite, injection site erythema

	and swelling, influenza-like symptoms. Uncommon or rare: hypersensitivity reactions including rash and urticaria, dizziness, nausea, abdominal pain, chills, dyspnoea. Very rare or reported post-marketing: anaphylaxis and angioedema, encephalitis, Guillain-Barre syndrome, and for Verorab sudden hearing loss which may persist. Causality is not established for the neurological events. Consult the current SPC for the product used.

Information for the patient

Written information	Supply the patient information leaflet for the product given, and a written record of the vaccine, the schedule used, the batch number and the date the next dose is due. Tell the patient to keep that record with their passport: if they are ever bitten, it determines what post-exposure treatment they need.
Counselling	● **THIS DOES NOT MAKE YOU IMMUNE TO RABIES. If you are bitten, scratched, or licked on broken skin by any mammal, you still need urgent medical treatment. What this does is buy you time and remove the need for rabies immunoglobulin, which is often unavailable where the risk is highest.** ● **Wash any bite or scratch immediately with soap under running water for several minutes, then seek medical help the same day, wherever you are.** ● **Come back for every dose. A part course is not a course. We have booked your next dose.** ● Avoid contact with animals while you are away, including dogs, cats, monkeys and bats. Do not feed or handle them. ● Some soreness, headache, aching or mild fever in the first day or two is common and settles on its own. ● Where the accelerated schedule was used: you will need a further dose at one year if you keep travelling to high risk areas.
Follow-up	Book the next dose of the course at this appointment. Seek urgent medical attention for any animal bite, scratch or lick on broken skin, whatever your vaccination status and however long ago. For a routine query about the vaccine or the schedule, contact the pharmacy.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- Rabies, Immunisation against infectious disease (the Green Book), chapter 27, dated 19 May 2023 (page last updated 26 May 2023). Summarised 9 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v008
Supersedes	Version 007, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
Valid from date	30 September 2026
Expiry date	31 July 2027

GET REAL HEALTH

Change history to version 004

Later versions are recorded in the Version and change record at the end of this document.

Version	v004
Date	9 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored, together with the standard governance requirements: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function that affected 15 live documents. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. This version changes no clinical content. ● Consent in children and young people added. Version 002 required only "valid informed consent obtained" while the conventional course has no lower age limit, so a child could be vaccinated with nothing in the document about parental responsibility or Gillick competence. Found by an estate-wide sweep of every vaccination PGD on 8 September 2026, which flagged the same gap in nine others. ● The accelerated day 0, 3 and 7 course is restricted to patients aged 18 and over. The document states plainly that this is a Get Real Health service restriction and not a guideline one, because Green Book chapter 27 sets no age limit and the Verorab evidence covers children from 2 years. The conventional course remains available at any age. ● The accelerated course is now identified as off-label, with a stated consent script and a requirement to record consent to off-label use naming the schedule. Neither product SPC contains a three dose day 0, 3 and 7 pre-exposure regimen; that schedule comes from the Green Book. This mirrors the handling of the accelerated Japanese encephalitis schedule. ● Immunosuppressed patients are excluded from the accelerated course, consistent with the Verorab SPC statement that its one-week regimen should not be used in immunocompromised individuals. ● A 15 minute observation period after every vaccination is stated in the document. Version 001 delegated it to the pharmacy SOP without giving a figure. ● A cold chain excursion procedure and a disposal provision have been added. Version 001 had neither. ● The differing dose volumes, Rabipur 1.0 mL and Verorab 0.5 mL, are called out as a check to be made before drawing up, and added to the training requirements. ● Latex in the Verorab prefilled syringe tip cap, and its phenylalanine content, added to the cautions. ● A note recording that Green Book chapter 27 predates the UK authorisation of Verorab, so its statement that Rabipur is the only UK-licensed intramuscular rabies vaccine is out of date. Both products in this PGD are UK-licensed. ● Records must now carry the product given, the schedule used and why, the dose number, the next dose date, and that the observation period was completed. ● The position on the two dose day 0 and day 7 SPC regimen is unchanged: it is not to be used under this PGD until UKHSA and JCVI guidance is published and this document is reissued.

Previous versions

002, 8 September 2026	Accelerated course restricted to 18 and over and identified as off-label with documented consent; immunosuppressed excluded from it; 15 minute observation, cold chain excursion and disposal added; differing dose volumes called out.
001, 21 August 2026	Standalone rabies pre-exposure PGD, split out of the combined Japanese encephalitis, rabies and meningococcal ACWY document following a query from Pharmacy Plus Health. Rabipur and Verorab, intramuscular route only.

Appendix 1: Key references

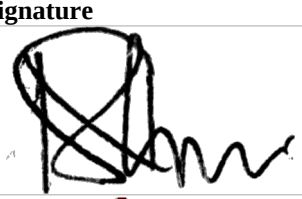
- Immunisation Against Infectious Disease (the Green Book), chapter 27, Rabies, 19 May 2023.
- UKHSA guidelines on rabies post-exposure treatment, current version, for the exposure pathway referred to under exclusions.
- Summary of Product Characteristics for Rabipur and for Verorab, current versions. Verorab PLGB 23228/0001, first authorised 1 November 2023, text last revised 29 January 2026.
- National Travel Health Network and Centre, travelhealthpro.org.uk, for country-specific rabies risk.
- UKHSA Vaccine Incident Guidance.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Version and change record
Version: v008
Issued: 30 September 2026
Supersedes: Version 007, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v007
Issued: 14 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The age floor is now stated the same way throughout: 2 years and over for the conventional course (the cover already said so; the inclusion row, the training row and the route row said any age or mentioned infants). Under 2 years is now listed as an exclusion, stated as a Get Real Health service restriction.
Neomycin hypersensitivity now excludes both products, since Rabipur contains trace neomycin as well as Verorab; the previous exclusion named Verorab only.
The unreachable sentence requiring parental consent to the accelerated course in a young person was removed, because the accelerated course is restricted to 18 and over.
The guidance summary now flags on its face that the chapter's statement that Rabipur is the only licensed vaccine is out of date, with a pointer to the Verorab note.
The title and the cover no longer read 'for the treatment of Pre-exposure Prophylaxis'.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 8 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.